
HAVEN PAEDIATRIC CENTRE

Building the systems under a hospital that already works

A conversation with the whole team about what changes, and what it means on your shift.

FOR

Everyone at Haven

FROM

Dr Debo Odulana, Consult for Africa

DATE

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Sixteen of seventeen of you rated the safety of care here very good or excellent.

That came from the anonymous survey, where you could have said anything at all and nobody would have known who said it. Seventeen of you answered, out of a team of nineteen, which is a response rate most hospitals never see.

We have worked with a lot of hospitals. A result like that is rare, and it is not luck. The hardest part of building a hospital, a team that genuinely cares, is already done here, and it was done by the people in this room before any of us arrived.

This is the thing we most want to protect. Everything else in a hospital can be rebuilt. That cannot, not quickly.



A good young hospital, growing faster than its systems were built for.

Haven is two years old. It has a real reputation and it is getting busier. What we are here to do is put proper systems underneath a hospital that has so far been held together by people caring enough to make it work anyway.

- This is not a criticism of anyone. It is what happens in every young hospital that is any good.
- The hospitals that skip this part are the ones that stop being good the moment they get busy.
- Culture is the multiplier. Everything we build works better because of what you already have.

We diagnose a system. We never diagnose a person. Nothing we have found is about anybody's performance, and nothing we do will be.



Three things, and all three are about forms and systems.

What we found	What it looks like to you	What we are doing
Work gets done and not written down properly, and the records do not always agree with each other	Writing the same thing in three places, and still being asked for it again	Rebuilding how work is recorded, so it is captured once
Stock runs out because there is no system telling anyone to reorder	Reaching for something and the shelf is empty	Proper reorder levels, and a supplier arrangement that keeps the shelf full
Almost nothing is written down as a standard, so the way care is given depends on who is on shift	Being told off for doing it the way you were taught	Writing the standards down, with you, not at you

The forms here were designed for a much smaller hospital than the one you are now running. That is the finding. We are going to fix the forms.



The test of all of this is whether your Tuesday gets easier.

What changes	What it means for you
Standards of work, written with the people who do the work	You stop being blamed for doing it the way you were taught
Stock and reordering fixed	The drug is on the shelf when you reach for it
The rota published four weeks ahead	You can plan your life
A proper handover and a short daily huddle	You find out what happened before you walk into it
Recording and billing rebuilt	Less paperwork chasing, not more
Job descriptions and targets reworked	What good looks like is written down, and it rewards care rather than activity



Most of what we are doing is about how this place works as a team.

The strongest thing Haven has is its people, so the biggest part of this work is about keeping that true as the hospital grows.

STANDARDS

Written down with you, one page each, kept where the work happens. We will start by asking you to show us how you do it now.

DEVELOPMENT

Structured induction for new people, training that is planned rather than improvised, and colleagues teaching colleagues.

RECOGNITION

The person who catches a near miss, or holds a difficult shift together, gets named for it. That costs nothing and it matters.



November to February is the busiest time of your year.

Everything we are doing now is so that you go into that season with the stock, the rota and the standards already in place, rather than finding out in December that they are not.

The pace over the next few weeks is not pressure on you. It is the work we have to finish before the hospital gets busy, so that when it does, the system carries some of the weight you have been carrying yourselves.

**Tired people are short with each other.
The season is exactly when the thing
we are protecting is most at risk.**



Worth saying plainly, because people always wonder.

- We are not here to find someone to blame. We diagnose systems, never people, and nothing we have found is about anybody's performance.
- We are not going to make you reconstruct records to make us comfortable. Partial or messy records are themselves a finding. Give us what exists.
- We will not change how you work without asking you first how you work now. That is the order, every time.
- Anything you tell us in confidence stays that way. Where we assess anyone individually, the result belongs to that person and goes to nobody else.



One thing. Tell us when something nearly went wrong.

Not so that anybody gets into trouble. A near miss is the cheapest lesson a hospital ever gets, and the hospitals that learn from them are the ones that stay safe when they get busy.

If reporting one ever costs somebody here anything, that is a failure of ours and we want to hear about it directly.

The second thing, if you will give us it: tell us what is broken. You know where this hospital leaks time and effort better than anybody who arrives with a clipboard.

You built the part that cannot be bought.

We are here to build the part that can, so that the thing you built survives this hospital getting bigger.



Tell us what is broken. We will be in the building.

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